CLINICAL TRIAL FAILURES · MISSED ENDPOINTS · IMMUNOLOGY & AUTOIMMUNE PHASE 2,3 · STARTS 2015–2024

BTK: 5 of 11 completed trials missed their primary endpoint

5 completed trials across 3 sponsors and 5 molecules ran to the end and missed their primary endpoint on the sponsor's own posted result; 4 met it. Only 3 of 26 closed trials were stopped early for efficacy or safety, which is why this class is read through its completed trials rather than a stop rate. Most completed trials post nothing readable, so the count is a floor.

5 of 11
completed trials with a readable primary result missed it · 4 met it

3 / 5
sponsors / molecules behind the misses

11.5%
stopped early: 3 of 26 closed trials (95% CI 4.0%–29.0%)

65.0%
of 40 trials have closed · 14 still open or unresolved

5 of 11 **Completed and missed the primary endpoint.** Of 22 completed trials in this segment, 11 posted a primary result on ClinicalTrials.gov that can be read — the sponsor's own comparison held to the sponsor's own threshold, or its statement that the endpoint was missed: 5 missed and 4 met, 2 split across co-primary endpoints. Counted separately and never in the rate above; a floor, not a rate. 4 of the 22 completed trials that finished more than a year ago have posted no results at all — a gap in what can be known, not a sign of failure.

The completed trials that missed

Trial	Ph	Sponsor	Experimental drugs	Started	What the posted result says
NCT04186871	2	Bristol Myers Squibb	BRANEBRUTINIB	2020-01	The Percent of Participants With mCLASI Response at Week 24 and Corticosteroid (CS) < 10 mg/Day at Week 20 and Week 24 - SLE — Response Difference -27.8 (95% CI -77.5 to 21.9), no p-value posted and no threshold stated: the interval is consistent with no effect under any conventional threshold, so not significant.
NCT03233230	2	Merck KGaA	M2591 25 mg QD, EVOBRUTINIB	2017-09	Percentage of Participants Who Achieved American College of Rheumatology 20 Percent (%) Response Criteria (ACR20) Assessed Using High-Sensitivity C-reactive Protein (hsCRP) at Week 12 — p 0.8283, no threshold stated; 0.2 or above, so not significant under any conventional threshold. Odds Ratio (OR) 1.06 (95% CI 0.61 to 1.87). Method: Regression, Logistic.
NCT02908100	2	Roche	FENEBRUTINIB	2017-01	Systemic Lupus Erythematosus Responder Index (SRI)-4 Response at Week 48 — p 0.373, no threshold stated; 0.2 or above, so not significant under any conventional threshold. Absolute Difference 6.4 (95% CI -8.5 to 21.2). Method: Cochran-Mantel-Haenszel.
NCT02784106	2	Merck KGaA	EVOBRUTINIB	2016-07	Proportion of Participants Who Achieved American College of Rheumatology-20 (ACR20) Response — Difference in Proportion of Responders 0.1 (80% CI -0.07 to 0.25), no p-value posted and no threshold stated: the interval is consistent with no effect under any conventional threshold, so not significant.
NCT02638948	2	Bristol Myers Squibb	BMS-986142	2016-02	Percentage of Participants Achieving American College of Rheumatology 20% (ACR20) Response at Week 12 — p 0.5224 against the sponsor's stated threshold of 0.05: not significant. Estimate of Difference (%) 4.9 (95% CI -10.2 to 20.1). Method: Chi-squared.

How this compares

BTK



11.5%

3/26 closed

all immunology & autoimmune trials with a resolved drug target



3.5%

30/869 closed

All immunology & autoimmune Phase 2,3



3.4%

68/1998 closed

What is in this class: 18 distinct experimental drugs across 40 trials (small molecule 40, unknown 1). Most tested: ZANUBRUTINIB (8), ORELABRUTINIB (6), RITLECITINIB (4), FENEBRUTINIB (4), ELSUBRUTINIB (4). A class groups drugs by what they act on; check that the grouping is one you would make before reading the rate as a property of the mechanism.

How much does this rest on one decision? The 3 stops came from 3 sponsor-asset programmes across 3 sponsors; the largest (Roche) contributed 1; removing that programme's trials from both sides leaves 9.1%. Screened alongside 28 other segments, the rate is within what screening that many classes could produce (q=1); the molecules and trials that failed here are the record either way. Read it as a screen worth checking against the underlying trials.

Bars show the share of closed trials stopped for biological reasons. Confidence intervals overlap where sample sizes are small — read the counts, not just the bars.

By start cohort

Trials started	Stops	Closed	Rate	95% CI
2015–2017	1	11	9.1%	1.6%–37.7%
2018–2020	2	13	15.4%	4.3%–42.2%

Where the stops sit

Efficacy / futility: 2, Benefit–risk: 1.
Sponsors with most stops: AbbVie (1), Roche (1), Eli Lilly (1).

The stopped trials

Trial	Ph	Sponsor	Experimental drugs	Stopped	Type	Registry stop reason
NCT03823378	2	AbbVie ABBV	ELSUBRUTINIB, UPADACITINIB	2020-09	Efficacy	Study M16-763 was terminated early as the benefit of each treatment arm from the feeder study (Study M16-063) did not provide appreciable evidence of differentiated clinical effect to warrant further long-term continuation.
NCT03407482	2	Roche	FENEBRUTINIB	2019-11	Efficacy	The study was ended early due to the lack of efficacy seen in the parent study GA30044.
NCT02628028	2	Eli Lilly LLY	POSELTINIB	2018-04	Benefit–risk	The trial was terminated after an interim analysis of efficacy and safety data did not warrant continuation of the trial.

Method
Denominator: ClinicalTrials.gov interventional Phase 2,3 immunology & autoimmune trials started 2015–2024 that have closed (completed or terminated). Numerator: terminated trials whose registry stop reason is classified as biological (efficacy, safety or benefit–risk) by Classification V2 — held-out precision 95.5%, recall 95.3% (n=600). Drugs are linked to ChEMBL and the NCI Thesaurus; 48% of industry immunology & autoimmune trials in this window carry a resolved drug target, and a trial without one cannot enter a mechanism class. Intervals are Wilson 95%.

Limits
Not a failure rate: completed trials that missed their endpoints are not in the rate. They are counted separately where the sponsor posted a primary result that can be read — most did not, so that count is a floor — and programmes discontinued after a completed trial do not appear. Stop reasons are sponsor-reported. This is a closed-trial proportion, not a time-to-event analysis: only 65.0% of trials in this segment have closed, and a trial that stops early enters the denominator sooner than one that runs to completion, which can inflate the rate in immature segments. Recent cohorts have fewer closed trials, so their rates are less stable. Research signals, not clinical or investment advice.

The whole cohort, trial by trial. The diligence report names every trial behind these figures — the ones still running and the ones that completed and missed their primary endpoint included — and places your molecule against each one that failed. Free during the beta: enter your work email at **clinicaltrialfailures.com/asset-check** and it opens straight away.

Sources: ClinicalTrials.gov (NLM); ChEMBL (EMBL-EBI, CC BY-SA 3.0); NCI Thesaurus (NCI); RxNorm/RxClass (NLM); SEC EDGAR. Classification, linkage and rates by Clinical Trial Failures. Rebuilt weekly; this brief covers trials started 2015–2024.